

DRIMINATE SUPPOSITORY 50MG

Ingredient(s)

Each suppository contains:

Dimenhydrinate 50mg

Pharmacology (Summary of Pharmacodynamic and Pharmacokinetic):

Pharmacodynamic

1. The action of dimenhydrinate is anti-histaminic (H₁ blocker) and is of the ethanolamine class with significant anticholinergic activity and sedative properties. The mechanism by which dimenhydrinate exert its antiemetic, anti-motion sickness, and antivertigo effects is not precisely known but may be related to its central anticholinergic action. Dimenhydrinate diminishes vestibular stimulation and depresses labyrinthine function. An action on the medullary chemoreceptive trigger zone may also be involved in the antiemetic effect.

Pharmacokinetic

2. Antiemetic effects occur within 30 to 45 minutes after rectal administration. The duration of action is 3 to 6 hours. Limited information is available on the distribution and metabolism of dimenhydrinate. Like other antihistamines, dimenhydrinate probably is widely distributed into body tissues, crosses the placenta, is metabolized by the liver, and is excreted in urine. Small amounts of dimenhydrinate are distributed into milk.

Indication(s):

For the prevention and relief of motion sickness and treatment of nausea, vomiting and vertigo.

Route of administration:

To be inserted into the rectum.

Dosage and Administration:

Adults: 1 – 2 suppositories every 6 to 8 hours.

Children: 8 to 12 years old: 1 suppository every 8 to 12 hours.

When dimenhydrinate is used for prophylaxis of motion sickness, it should be inserted at least 30 minutes (preferably 1 to 2 hours) before travelling.

To be inserted into the rectum.

Contraindication(s):

1. Patients with known sensitivity to dimenhydrinate or diphenhydramine.
2. Use is not recommended in nursing mothers and in newborn or premature infants.

Side Effect(s) / Adverse Reaction(s):

Drowsiness commonly occurs after administration of dimenhydrinate. Paradoxical CNS stimulation may occur in children and occasionally in adults. Other adverse effects include headache, blurred vision, tinnitus, dryness of the mouth and respiratory passages, incoordination, palpitation, dizziness, and hypotension. Anorexia, constipation or diarrhea, urinary frequency, and dysuria are less common.

Precaution(s) / Warning(s):

1. Dimenhydrinate may lead to drowsiness and impaired concentration, which may be aggravated by simultaneous intake of alcohol or other central nervous system depressant agents. Patients should be warned not to drive or operate machinery. Effect on ability to drive and use machine
2. Caution is recommended when dimenhydrinate is used since its antiemetic action may impede diagnosis of such conditions as appendicitis and obscure signs of toxicity from overdosage of other drugs.
3. Dimenhydrinate should be used with care in medical conditions such as bladder neck obstruction, prostatic hypertrophy, urinary retention, glaucoma and epilepsy.
4. Use is not recommended in newborn or premature infants because this age group has an increased susceptibility to anticholinergic side effects, such as CNS excitation, and an increased tendency toward convulsions.

Interaction with Other Medicaments

- a. Dimenhydrinate may enhance the effects of other CNS depressants such as alcohol and barbiturates. If dimenhydrinate is

used concomitantly with other CNS depressants, caution should be used to avoid overdosage.

- b. Dimenhydrinate may potentiate the effects of other drugs with anticholinergic activity including tricyclic antidepressants and MAOIs.
- c. When given concurrently with aminoglycoside antibiotics or other ototoxic drugs, dimenhydrinate may mask the early symptoms of ototoxicity.

Pregnancy and Lactation:

Reproduction studies in rats and rabbits using dimenhydrinate doses up to 20 and 25 times the human dose (on a mg/kg basis) respectively have not revealed evidence of harm to the fetus or impaired fertility. There are no adequate and well-controlled studies using dimenhydrinate in pregnant women. Clinical studies to date in pregnant women receiving the drug have not indicated an increased risk of abnormalities when administered during any trimester. Although the possibility of harm to the fetus appears remote, dimenhydrinate should be used during pregnancy only when clearly needed.

Small amounts of dimenhydrinate are distributed into milk. Because of the potential for adverse reactions to dimenhydrinate in nursing infants, a decision should be made whether to discontinue nursing or the drug, taking into account the importance of the drug to the woman.

Symptoms and Treatment for Overdosage, and Antidote(s):

Overdose is associated with anticholinergic, extrapyramidal, and CNS effects. In infants and children, CNS stimulation predominates over CNS depression, causing ataxia, excitement, tremors, psychoses, hallucinations, and convulsions; hyperpyrexia may also occur. Deepening coma and cardiorespiratory collapse may follow. In adults, CNS depression is more common with drowsiness, coma, and convulsions, progressing to respiratory failure and cardiovascular collapse.

Since there is no specific antidote for overdose, treatment is symptomatic and supportive.

To decrease absorption:

- Induction of emesis (syrup ipecac recommended); however, precaution against aspiration is necessary, especially in infants and children.
- Gastric lavage (isotonic or 0.45% sodium chloride solution) if patient is unable to vomit within 3 hours of ingestion.

To enhance elimination:

- Saline cathartics (milk or magnesium) are sometimes used.

Specific treatment:

- Vasopressors to treat hypotension; however, epinephrine should not be used since it may further lower blood pressure.
- Oxygen and intravenous fluids

Precaution against use of stimulants (analeptic agents) because they may cause seizures.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Store at temperature below 25°C. Protect from light and moisture.

Product Description and Packing(s):

A white to off-white suppository.

10 supp. per strip, 10 strips per box.



Y. S. P.

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